National Cancer Control Programme (NCCP) 00733 · clinician draft

Fortnightly cetuximab + modified FOLFOX-6

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1 · chemotherapy and antibody

Active medicines: Cetuximab + Oxaliplatin + Folinic acid + Fluorouracil

Emetogenic risk: Moderate

Medicine	Dose / route	When
Chlorphenamine	10 mg intravenous (IV) bolus	At least one hour before cetuximab Repeat for subsequent cetuximab infusions; assess prior infusion reactions.
Dexamethasone	8 mg IV bolus	At least one hour before cetuximab on Day 1 This corticosteroid also covers the treatment-day corticosteroid part of antiemesis where appropriate. Pharmacy must reconcile route, dose and timing; do not automatically add a second 8 mg antiemetic dose.
Ondansetron	16 mg PO once	Before Day 1 chemotherapy NCIS V8 moderate-risk row; consider QT risk, low electrolytes, interactions and the individual prescription.
Dexamethasone	8 mg PO daily	Days 2 and 3 after the administered Day 1 chemotherapy NCIS V8 delayed prophylaxis. Reconcile with any other steroid prescription; re-anchor dates if treatment is delayed.
Metoclopramide	10 mg PO up to three times daily as required	Rescue nausea treatment only as prescribed NCIS V8 rescue option; check contraindications, renal function, interactions and prescribed duration.

Additional phase-specific support

- Antiemetic classification refers to the active treatment day. Reconcile duplicate steroid entries before prescribing.
- No automatic granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) schedule is added; use the patient's neutropenia history, risk and consultant plan.
- Cetuximab premedication and infusion observation remain necessary even on an antibody-only day.
- Start local EGFR skin care; monitor magnesium, calcium and potassium before and periodically during therapy, then monthly for two months after the final dose.
- Cold exposure can precipitate oxaliplatin neurotoxicity; do not recommend ice chips during fluorouracil in this combination. Review neuropathy every two cycles and sooner if symptoms interfere with function.
- Confirm line care, pump contact details and disconnection arrangements; report chest pain, severe early fluorouracil toxicity or pump/line problems urgently.
- No ice chips during fluorouracil in this oxaliplatin regimen. Review neuropathy and cold-triggered throat symptoms; new unexplained cough or breathlessness requires urgent assessment.
- If treatment is delayed for cytopenia, follow the source-specific weekly blood-count checks and maximum delay; assess the antibody and chemotherapy components separately when indicated.
- EMA Erbitux section4.2: chemotherapy must not begin earlier than one hour after the end of cetuximab.
- Observation attribution: NCCP permits stopping routine post-infusion observation after two tolerated doses; EMA Erbitux specifies close monitoring during and for at least one hour after infusion. Reconcile the approved local observation policy; do not automate removal of monitoring.
- Medication review: NCCP contraindicates concomitant brivudine, sorivudine and analogues with fluorouracil; review dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD)-affecting medicines, phenytoin toxicity/levels and warfarin international normalised ratio (blood-clotting test) (INR).

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00733 v2a · p6 · NCCP NCIS antiemetics V8 (2025), Table 1 and multi-day principles

Sources checked 2026-11-05 · SACTCheck v0.84.35 · Regimen-level drafts; prescriber and oncology-pharmacy verification required